

Kynesis: Business Plan

1. The Problem and Why It's Real

Progressive conditions like Parkinson's, Huntington's, dementia, and Alzheimer's require continuous monitoring because movement itself signals disease progression. Currently, patients are only evaluated during clinical appointments spaced weeks or months apart, leaving a massive gap in observation. In countries where specialists are scarce, this gap is critical.

Existing solutions either require patients to purchase expensive wearables (like Apple Watches) which have low adoption rates, or they rely on subjective self-reporting apps that depend on the patient's memory. Kynesis solves this by turning a device patients already own—their laptop—into a clinical tracking instrument. By having patients play short tasks using just their keyboard and trackpad, Kynesis objectively measures actual movement and compares it against the patient's personal baseline over time, generating real data for both the patient and their doctor.

2. Projected Target Audience and Market Size

Kynesis targets one of the largest untapped patient populations for digital health products, focusing on individuals already diagnosed and receiving treatment but lacking between-visit monitoring. The Indian market size includes:

- **Parkinson's Disease:** 2 to 5 million patients. Nearly half are diagnosed with early-onset (ages 22–49), meaning a significant portion are of working age and highly comfortable with daily laptop use.
- **Alzheimer's and Dementia:** 8.8 million patients. This represents a massive scale lever that continues to grow alongside the aging population.
- **Huntington's Disease:** 20,000 to 40,000 patients.

3. Costs and Cost Scaling

Because Kynesis is a purely software-based product, there is zero hardware to manufacture, store, or ship.

- **Primary Costs:** The core expenses are the team itself, specifically engineering (to build and maintain the platform) and a small clinic-partnerships team (to onboard and support clinics).
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- **Scaling Economics:** As user adoption grows, the cost per patient remains near zero because adding new users simply means provisioning new accounts on the existing system. The only cost that scales with growth is the expansion of the partnerships team required to manage larger hospital accounts, but this expense scales much slower than the resulting revenue.
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4. Sources of Revenue and Revenue Scaling

The business operates on a B2B2C model with multiple revenue streams designed to scale from pilot phases to enterprise-level contracts:

- **Clinic Licensing (Year 1+):** Clinics pay a yearly fee of roughly ₹150–400 per patient per month. Year 1 focuses on 150–250 patients across pilot clinics (₹1–3 lakhs) to establish proof of concept.
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- **Family Subscriptions (Year 2+):** Families pay a monthly premium (₹199–499) for peace of mind to monitor a relative's condition. Assuming a 20% conversion rate at scale, this generates an estimated ₹15–17 lakhs annually.
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- **Hospital Enterprise Contracts (Year 2+):** Hospitals pay flat yearly licensing fees (₹5–20 lakhs per hospital) to deploy the software across entire neurology departments.
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- **Anonymized Data Licensing (Year 3+):** Once the platform has longitudinally tracked thousands of consenting patients, the anonymized dataset becomes highly valuable to pharmaceutical companies recruiting for specific disease stages in clinical trials. This represents the highest-margin revenue stream.
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Revenue Projections

Phase	Estimated Revenue	Focus
Year 1	₹1 Lakh – ₹3 Lakh	Proving the concept, acquiring pilot clinics, research at scale.
Year 2	₹3 Lakh – ₹10 Lakh	Expanding clinic reach, introducing family subscriptions.
Year 3	₹10 Lakh – ₹20 Lakh	Securing larger hospital deals, expanding user base, improving margins.

5. Plans for Growth

- **Step 1: Prove it works (Parkinson's focus):** Launch initially in 2–3 movement-disorder clinics. Parkinson's is the starting point because the scientific link between the disease and motor output is the strongest, providing a clear proof of concept.
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- **Step 2: Expand to the other three conditions:** Reuse the existing platform engine with adjusted scoring and minor task additions (e.g., word-finding tasks for dementia). Expanding to the 8.8 million+ Alzheimer's/dementia market acts as the primary scale lever.
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- **Step 3: Widen Distribution:** Transition free pilot clinics into paying accounts, actively market family subscriptions, and leverage the generated clinical data to close lucrative enterprise hospital contracts.
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- **Step 4: Data Asset & Geographic Expansion:** Monetize the longitudinal dataset with pharma partners. Long-term, expand the "no extra hardware" model to regions like Southeast Asia and Africa where wearable penetration remains similarly low.